Nephrology Clinical Assessment Report

Chronic Kidney Disease Screening · Confidential Clinical Document

Patient Information

Patient Name	Robert H.	**Patient ID**	CKD-2026-001
Date of Report	April 29, 2026	**Exam Type**	Nephrology Screening — CKD Assessment
Age / Gender	68 years · Male	**Referring Department**	Internal Medicine
Hypertension	Yes — currently managed	**Diabetes History**	Not reported
Prior CKD Diagnosis	Not formally confirmed	**Informant**	Patient / Medical Record

Laboratory & Clinical Findings

Parameter	Result	Reference Range	Status	Clinical Note
Hemoglobin	**7.5 g/dL**	13.5–17.5 g/dL	**LOW**	Renal anemia — significant oxygen-carrying deficit
Specific Gravity	**1.008**	1.010–1.030	**VERY LOW**	Loss of urinary concentrating ability — tubular dysfunction
Albumin (urine)	**3+**	Negative / Trace	**HIGH**	Severe proteinuria — significant glomerular damage
Packed Cell Volume	**24%**	38.3–48.6%	**LOW**	Hematocrit critically reduced — secondary anemia
Serum Creatinine	**4.2 mg/dL**	0.7–1.2 mg/dL	**VERY HIGH**	Severe GFR impairment — advanced kidney failure
Hypertension	**Present**	Absent	**FLAGGED**	Major independent risk factor for CKD progression
Age	**68 years**	—	**RISK**	Advanced age elevates baseline nephropathy risk

5 out of 5 quantitative markers deviate significantly from reference ranges. All abnormalities are directionally consistent with advanced chronic kidney disease.

Clinical Observations

Robert H. is a 68-year-old male referred from Internal Medicine for nephrology assessment. The patient has a confirmed history of hypertension, currently under pharmacological management. No formal prior diagnosis of chronic kidney disease (CKD) is on record, though the current laboratory profile is strongly suggestive of advanced, long-standing renal dysfunction. The patient reports fatigue, reduced urine output, and lower limb edema over recent months. No history of diabetes mellitus has been reported, though this has not been formally excluded. The combination of his age, hypertensive history, and the severity of the laboratory findings represents a high-risk nephrology profile.

Interpretation of Key Findings

Serum Creatinine (4.2 mg/dL): Severely elevated. At this level, estimated GFR (eGFR) falls below 15 mL/min/1.73m², placing the patient in CKD Stage 5 (kidney failure) territory. Immediate nephrology intervention is indicated.

Albumin (3+): Severe proteinuria reflects extensive glomerular damage and is a strong independent predictor of CKD progression and cardiovascular mortality.

Specific Gravity (1.008): A consistently low specific gravity indicates the kidneys have lost their concentrating ability — a hallmark of tubular dysfunction in advanced CKD.

Hemoglobin (7.5 g/dL) and PCV (24%): Both values indicate severe anemia of chronic kidney disease, likely driven by reduced erythropoietin production. Symptomatic anemia at this level warrants urgent intervention.

Hypertension: A well-established bidirectional risk factor — both a cause and consequence of CKD. Blood pressure management is critical to slowing disease progression.

Clinical Impression

The clinical and laboratory profile of Robert H. is consistent with Chronic Kidney Disease at an advanced stage, most likely Stage 4–5 based on the degree of creatinine elevation and estimated GFR. The convergence of severe proteinuria, critically reduced hematological indices, impaired urinary concentrating capacity, and uncontrolled hypertensive burden constitutes a high-confidence clinical picture of end-stage renal disease progression. Urgent nephrology follow-up and preparation for renal replacement therapy evaluation are strongly indicated. This report does not substitute for a formal clinical diagnosis.

Recommendations

1. Urgent nephrology referral for formal CKD staging and eGFR calculation using CKD-EPI equation.
2. Initiate or intensify erythropoiesis-stimulating agent (ESA) therapy for symptomatic renal anemia; consider iron supplementation assessment.
3. Optimize antihypertensive regimen — target BP <130/80 mmHg; ACE inhibitors or ARBs preferred for renoprotective effect in proteinuric CKD.
4. Dietary nephrology consultation: protein restriction, phosphate and potassium management.
5. Evaluate candidacy for renal replacement therapy (hemodialysis, peritoneal dialysis, or transplantation) given the proximity to Stage 5 CKD.
6. Screen formally for diabetes mellitus (HbA1c, fasting glucose) as an underlying etiology.
7. Cardiovascular risk stratification — severe CKD and proteinuria significantly elevate CV event risk.

Examining Clinician — Nephrology
Date: April 29, 2026